ONE Clinical Low Vision Protocol

POSSIBLE F-TAG/QMS

Survey Regulations

- F 676- Maintain ADL, Mobility, and Communication Abilities
- F684-Maintain Physical, Mental, and Psychosocial Well Being
- F 685- Maintain Vision and Hearing abilities

Quality Measures

- Falls with Major Injury
- Help with ADLs
- Ambulation
- Expected DC Score
- Safe Community DC
- ER Visits
- Hospitalizations

THERAPIES

- PT
- OT
- ST

PURPOSE OF THERAPY

- Train patients and caregivers on use of equipment and strategies to maximize use of patient available vision for daily self-care tasks, activity engagement, and mobility to improve quality of life.
- Low vision therapy benefits people with diagnoses such as macular degeneration, diabetic retinopathy, retinitis pigmentosa, and glaucoma. In general, low vision cannot be corrected with glasses, surgery, or medication.

GOAL EXAMPLES

- Patient will utilize filters with therapist to view contrast sensitivity chart at 10% to decrease risk for falls 5/5 treatment sessions. (1.25% is highest/best on chart)
- Patient will utilize filters (without assistance/with trained caregiver) to view contrast sensitivity chart at 5% to decrease risk for falls 5/5 treatment sessions.
- Patient will utilize stand magnifier and/or adaptive strategies to read personal documents/prescription information/cooking instructions with 75% accuracy to improve safety and effectiveness with IADLs.
- Patient will utilize stand magnifier and/or adaptive strategies to read personal documents/prescription information/cooking instructions with 100% accuracy to improve safety and effectiveness with IADLs.
- Patient will utilize eccentric viewing strategies with therapist to accommodate for central scotoma and decrease risk for falls 5/5 treatment sessions.
- Patient will utilize eccentric viewing strategies (without cues/with trained caregiver) to accommodate for central scotoma and decrease risk for falls 5/5 treatment sessions

TOOLS, ASSESSMENTS & IMPLEMENTATION

TOOLS & ASSESSMENTS

PCC Assessments:

- Identify residents with Macular Degeneration, Diabetic Retinopathy, Retinitis Pigmentosa, Glaucoma Diagnoses
- (Diagnosis Report-Select Date Range-Select Code-All Related Diagnoses-Run Report)
- **MNREAD Acuity Chart**
- **Visual Field Tests**
- **Low Contrast Reading Charts**

INTERVENTIONS

- Color Filter Trials
- Stand and Handheld Magnifiers
- Raised Adhesives for Tactile Identification
- Phone Accessibility (Visual & Audio Options)
- Phone Magnification
- Lighting Assessments
- Eccentric Viewing Strategies
- Environmental Modification (High Contrast)
- Low Vision ADL & IADL Aids

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POSSIBLE F-TAG/QMS	THERAPIES	PURPOSE OF THERAPY	GOAL EXAMPLES	TOOLS, ASSESSMENTS & IMPLEMENTATION
			• Patient will utilize strategies (eccentric viewing, magnifier, phone magnification, adapted lighting) to read 2.0 M size print to improve ability to read medicine instructions and important personal documents. • Patient will utilize strategies (eccentric viewing, magnifier, adapted lighting) to read 0.8 M size print to improve ability to read medicine instructions and important personal documents. • Patient will utilize vision strategies with therapist/trained caregiver to consume 100% of PO intake during meal. • Patient will utilize scanning strategies with therapist/trained caregiver during mobility tasks for 4/5 sessions to decrease risk for falls. • The patient will utilize visual aids and vision strategies for call light assistance provided during transfers in 10/10 attempts to decrease risk for falls. • The patient will sequence 5/5 steps to place a call to spouse on cell phone, with use of visual sequence aid and low vision strategies to increase ability to make phone calls to family/friend to communicate daily wants/needs. • The patient will recall 2/3 daily scheduled events for 2 consecutive sessions using low vision aids to increase schedule management and decrease resistance to care and behaviors.	